

C14 EMERGENCY RESOURCES | O (MAX : 2 PT)

Intent : Provide resources, personnel and training to help organizations, families and individuals respond to diverse emergency situations.

Summary : This WELL feature requires projects to offer resources like first aid kits, automated external defibrillators (AEDs) and opioid response kits, coordinate with emergency response teams and provide emergency preparedness and response trainings.

Issue : It is estimated that sudden cardiac arrest (SCA) causes between 6.8 – 8.5 million deaths worldwide per year, with a global survival rate of less than 1%; in the United States, about 10,000 SCA deaths per year occur in the workplace.^{1,2} An SCA victim's chances of survival lower by 7-10% with every minute that passes without cardiopulmonary resuscitation (CPR) or defibrillation.³ Additionally, nearly 16,000 people worldwide die from preventable injuries each day, yet in most European countries, only 5-10% of the population is trained in first aid.^{4,5} While natural disasters kill an average of 90,000 people annually, nearly 60% of American adults have not practiced what to do in a disaster.^{6,7} Moreover, anaphylaxis causes up to 1,500 deaths per year in the U.S., with studies showing a delay in administering epinephrine to be a significant risk factor associated with fatal outcomes from allergen exposure.⁸⁻¹⁰ Finally, drug overdose is the leading cause of accidental death in the U.S., with overdose from opioids driving the epidemic.¹¹

Solutions : Rapid and effective emergency response requires coordination with local emergency responders and maintenance of emergency resources such as an emergency notification system, first aid kits and AEDs.^{12,13} Supplementing those resources with occupant training on CPR, first aid, AED use and individual and family preparedness can increase individual response time and help improve survival rates; CPR and AED training alone can increase victim survival rates by nearly 40%.^{3,12} In food allergy emergencies, quick access to and immediate availability of epinephrine is essential.^{10,14} Finally, increasing the availability of naloxone is a critical component of reducing opioid-related overdose deaths, with evidence suggesting that when naloxone and overdose education are available to community members, overdose deaths decrease in those communities.¹⁵

PART 1 PROMOTE EMERGENCY RESOURCES (MAX : 1 PT)

For All Spaces:

1: Emergency resources

Resources are in place that support emergency response, including at least three of the following:

- a. Information indicating emergency procedures (e.g., evacuation during fire or earthquake, containment and response strategies for infectious disease outbreaks, shelter-in-place during active shooter) available to all guests upon entrance to the building.
- b. Building emergency notification system with auditory and visual indicators of emergency (e.g., public address systems, flashing lights).
- c. At least one first aid kit per floor meeting requirements of Appendix C3.
- d. AEDs accessible to any occupant within 100 meters and adoption of routine maintenance and testing schedule.^{16,17,18} The locations of building AEDs are identified through posters, signs or other forms of communication other than on the AED itself.¹⁷
- e. Undesignated epinephrine auto-injectors for food allergy emergencies.¹⁴
- f. Rides for employees subsidized or reimbursed by at least 50% to destination of need for emergency situations (e.g., urgent medical needs, personal or family emergency), including from home to work as needed (e.g., during public transit shutdown).

2: Emergency training and personnel

At least two of the following are in place:

- a. Emergency response team for medical emergencies, including at least one certified medical professional, first responder or other qualified personnel who has received emergency medical training (e.g., Emergency Medical Technician, paramedic, police, fire service, individuals certified in advanced first aid) present within the building during regular business hours.^{17,18}
- b. Security or crisis response team for human-caused disruptions (e.g., active shooter, civil unrest).
- c. Annual availability to regular occupants of a certified training course on CPR, first aid and AED usage.¹⁷
- d. Trainings to promote emergency preparedness available to regular occupants that address at least the following topics:
 1. Creating evacuation or sheltering plans.
 2. Building emergency kits, supplies and go-bags.
 3. For residents, if applicable, planning communications with family or primary contacts in case of emergency.

PART 2 PROVIDE OPIOID RESPONSE KIT AND TRAINING (MAX : 1 PT)

For All Spaces:

1: Opioid response kits

The following requirements are met:

- a. All emergency preparedness or first aid kits include:
 1. Naloxone rescue kits. Projects may choose a single dose nasal spray, a multi-step nasal spray, a single step injection or a multi-step injection.
 2. Instructions for how to prepare and administer naloxone, as well as immediate next steps after administration.
 3. A list of who on-site has received opioid response training, and their contact information.

- b. Protocol is in place for follow-up after an opioid emergency event, including a plan for:
 1. Debriefing those affected.
 2. Immediate replacement of naloxone kit following use.
 3. Replacing expired kits.

2: Opioid response training

The following requirement is met:

- a. Regular occupants receive opioid emergency training in-person or virtually, covering:
 1. General information about opioid use and naloxone.
 2. Recognizing the signs of an overdose and immediate steps to take.
 3. How to safely administer naloxone and steps to take following administration.

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